

- Assess wounds/ulcers for signs of infection
- Daily moisturizers for eczema
- Teach client how to correctly apply compression stockings
- Teach proper skin care
- Teach client to avoid staying in one position for long periods; leg exercises
- Teach client to elevate their legs (above heart)

- * Usually caused by varicose veins or complication of DVT
- * Fluid/RBCs leak into tissues = Brown discoloration

pathophysiology

Nursing Implications/Patient ed.

Chronic Venous Insufficiency

Risk Factors

Assessment/Clinical Manifestations

Collaborative Care

Labs/Diagnostics

Complications

Assess skin/circulation:

- Lower leg skin: leathery
- Brownish appearance
- Edema present
- Eczema/itching
- painful venous ulcers
- Adequate cap refill
- Skin warm
- Pulses present; may be difficult to palpate due to edema
- ulcers: irregular shape, yellow slough or ruddy granulation
- pull ache pain in leg

- Duplex ultrasound with Doppler
- CT venogram/venogram

- Venous ulcers
- osteomyelitis
- malignant changes
- Amputations (rare)

- 3E's
 - Elevation
 - Exercise
 - Elastic stockings
- Moist dressing
- Nutritional Therapy:
 - High in protein
 - vitamins/supplements
 - Tight glucose control for DM patients
- Antibiotics for wound infection
 - Systemic antibiotics
 - should be guided by wound culture results
- wound debridement and excision